

NHS Devon Integrated Care Board Service Specification

Service Specification No.	
Service	Refugee and People Seeking Asylum - GP Registration and Claiming Process
Commissioner Lead	Lauren Benham – Head of Health Inequalities
Provider Lead	GP practices
Period	01/04/2026-31/03/2029 (3 months' notice will be required to terminate this contract by either party)
Date of Review	Annual review re inflation Formal LES review: October 2028

1. Population Needs

1.1 National/local context and evidence base

Refugees and Asylum seekers face many of the same health problems as the UK population. In addition, they may:

- Have limited understanding of how the NHS operates and face multiple barriers to accessing treatment
- Come from countries of origin with poor healthcare
- Suffer health impacts (mental and physical) after leaving their country and being detained in the UK
- Have experienced war, conflict, violence and/or torture as well as other complex traumas
- Be separated from family, have poor housing and available nutrition, a lack of autonomy or certainty about their future, and be socially isolated.

This service aims to provide refugees and asylum seekers, no matter their country of origin, with fair and equal access to primary medical services. The service will provide both the individual and the local GP Practice where the individuals are registered with additional support to be able to manage this patient group.

Patients that are temporarily housed in the hotels should have their health needs generally considered in the same way that those of primary medical care permanent residents plus uplift in service provision to reflect the public health and acute care needs of this vulnerable patient cohort.

Refugee

A refugee is defined by the [1951 UN Convention Relating to the Status of Refugees](#) as someone who owing to well-founded fear of being persecuted for reasons of race, religion, nationality, membership of a particular social group or political opinion, is outside the country of [their] nationality and is unable or, owing to such fear, is unwilling to avail [themselves] of the protection of that country; or who, not having a nationality and being outside the country of [their] former habitual residence, is unable or, owing to such fear, is unwilling to return to it.

Asylum seeker

An asylum-seeker is someone who is seeking international protection. Their request for refugee status, or complementary protection status, has yet to be processed, or they may not yet have requested asylum but they intend to do so. While they seek asylum and await the outcome of their application, they are referred to as asylum-seekers and should be protected. Not all asylum-seekers

will be found to be refugees, but all refugees were once asylum-seekers.

2. Outcomes

2.1 NHS Outcomes Framework Domains & Indicators

Domain 1	<u>Preventing people from dying prematurely</u> Reducing deaths in babies and young children	X
Domain 2	<u>Enhancing quality of life for people with long-term conditions</u> <u>Health related quality of life for people with long term conditions</u> <u>Ensuring people feel supported to manage their condition</u> <u>Improving functional ability in people with long term conditions</u>	X
Domain 3	<u>Helping people to recover from episodes of ill-health or following injury</u> Emergency admissions for acute conditions that should not usually require hospital admission	X
Domain 4	<u>Ensuring people have a positive experience of care</u> Patient experience of primary Care Patient experience of hospital care Friends and family Test <u>Improving children and young people's experience of healthcare</u> <u>Improving peoples experience of integrated care</u>	X
Domain 5	<u>Treating and caring for people in safe environment and protecting them from avoidable harm</u> Patient safety Incidents reported Reducing the incidence of avoidable harm	X

3. Scope

3.1 Aims and objectives of service

The purpose of this agreement is to clarify arrangements between NHS Devon and GP practices which have agreed to accept and register new patients who are permanently or temporarily resettling as refugees and asylum seekers. This may include unaccompanied children.

The agreement outlines expectations for GP Practices which will be receiving a one-off funding payment of £181.30 (risen from an initial tariff of £150 in line with inflation) per person (in addition to usual capitation and contractual payments) following a patient registration in recognition of the additional time required to identify health needs, and to plan and deliver an optimum level of primary health care in response to these. The agreement also sets out the payment arrangements, support and guidance which will be provided by the ICB and other local partners.

This service is aimed at all patients resettling as refugees and asylum seekers, located within a practice's boundary.

3.2 Service description/care pathway

GP Practice Requirements

- Practices will undertake an initial detailed, extended medical assessment of the new patients. The national specification states an invitation to register to be sent within 10 days of arrival and a health assessment to have taken place within 30 working days of registration in order to identify what treatment or care they may need to receive, at present or in the near future. The assessment is carried out in the context of the services to be delivered as per the GMS/PMS/APMS contract.
- Ensuring a clear understanding of refugees' and asylum seekers health and wellbeing needs is vital for the delivery of health care to meet their needs and to contribute to their successful resettlement. This comprehensive health assessment will allow for a more in-depth assessment of their health status and needs than is currently covered in the standard new patient check and can cover anticipated issues such as immunisation status, psycho-social issues, eyesight needs. An appropriate amount of time must be allocated for an initial assessment per person. Men and women should be seen separately to allow discussion on potentially sensitive issues. Children should be seen with parent/s.
- The language abilities of refugees and asylum seekers must be recognised and appropriate interpretation planned for accordingly, using formal, rather than informal, ideally face to face interpreting services(i.e. not family members, friends etc.)
- Snomed codes must be used to flag refugees and asylum seekers on clinical record.

For further information and guidance:

[NHS entitlements: migrant health guide - GOV.UK \(www.gov.uk\)](http://www.gov.uk)

[Migrant health guide - GOV.UK \(www.gov.uk\)](http://www.gov.uk)

[Immunisation: migrant health guide - GOV.UK \(www.gov.uk\)](http://www.gov.uk)

[Assessing new patients from overseas: migrant health guide - GOV.UK \(www.gov.uk\)](http://www.gov.uk)

[Children's health: migrant health guide - GOV.UK \(www.gov.uk\)](http://www.gov.uk)

[Culture, spirituality and religion: migrant health guide - GOV.UK \(www.gov.uk\)](http://www.gov.uk)

[Tuberculosis \(TB\): migrant health guide - GOV.UK \(www.gov.uk\)](http://www.gov.uk)

[Tuberculosis \(TB\) screening for the UK: leaflet - GOV.UK \(www.gov.uk\)](http://www.gov.uk)

[Diphtheria: public health control and management in England - GOV.UK \(www.gov.uk\)](http://www.gov.uk)

Participating GP practices are strongly encouraged to sign-up to Doctors of the World [Safe Surgeries](#) initiative and promote [GP access cards](#). [Doctors of the World is a valuable source of translated materials which can be found here](#)

3.3 Population covered

Any persons requiring an initial health assessment upon GP registration, unless screening has previously been carried out and does not require an update. Individuals will be formally identified by the Devon system as being a refugee or a person seeking asylum, including unaccompanied children (as defined within section 1.1), and be located within a practice's boundary.

3.4 Any acceptance and exclusion criteria and thresholds

Acceptance criteria as per point 3.3.

Refugees, asylum seekers and refused asylum seekers can register for and receive primary care free of charge in the same way as any other patient in any nation of the UK. This is because immigration and residency status have no bearing on a patient's entitlement to register with a GP practice.

Patients can be registered without having to provide any documentation, proof of identity or

immigration status (though asylum seekers may have an 'application registration card' from Immigration Services).

They can also be registered without permanent housing. This is as a temporary patient for up to 3 months.

3.5 Interdependence with other services/providers

Other Primary Care services, Secondary Care and Social Care. Please refer to the below for further details:



service map 3.pdf

4. Applicable Service Standards

4.1 Applicable national standards (eg NICE)

The provider is required to adhere to all national standards as issued from time to time by any relevant Regulatory and Statutory bodies including guidance issued by appropriate competent bodies (e.g. Royal Colleges)

5. Applicable quality requirements and CQUIN goals

5.1 Applicable Quality Requirements (See Schedule 4 Parts [A-D])

Each practice must ensure that any personnel involved in providing any aspect of care under this scheme has the necessary training, skill and competency to do so.

5.2 Applicable CQUIN goals (See Schedule 4 Part [E])

Not applicable.

6. Location of Provider Premises

The practice name and address as specified in the primary medical services contract. The Practice may also choose to deliver assessments and clinics on-site for residents of asylum seeker hotels if an appropriate clinically safe space is available. Practices choosing to provide services on-site should ensure that the CQC are notified of these changes.

7. Funding arrangements

GPs accepting scheme participants will receive a one-off payment of £181.30 per person, in addition to usual contractual funding e.g. capitation. Claiming will take place via the ICB's commissioning intelligence portal.

[The ICB reserves the right to take this into account when negotiating any payments e.g. for atypical practices in relation to refugees and asylum seekers; and, to renegotiate this specification and/or fee giving a minimum 3 months' notice should the need arise].

DDRB will be used as the start point for annual 'inflationary' review, with any deviation from DDRB uplift requiring clear rationale.

In the event of a pandemic or other 'level 4 health' event, commissioner and LMC will meet urgently to review this specification giving consideration to:

- a. suspension of some or all elements of it,
- b. redirection of patients to alternative provider(s),
- c. in the event of actioning either a) or b) determining appropriate income protection arrangements for GP provider